

GSK PLC

Core EBIT margin target implies c. 1% upside to 2030
Consensus, pipeline opportunities highlighted
encouraging, though still early with clinical data required
to drive upgrades

Yesterday, we attended GSK's 2Q 2026 and "Accelerate Growth" event (see our first take [here](#)). We summarise our takeaways below. GSK reiterated their confidence in the >£40bn 2031 Sales target, which now includes the contribution from Nuvalent, and outlined their restructuring program expected to generate £1.9bn in annual savings by 2029, the majority of which is expected to be reinvested in R&D and business development. We estimate a potential c. £200m benefit to Core EBIT by 2029 from these savings, a c. 60bps Core EBIT margin benefit although note that Consensus already models a 40bps increase in Core EBIT margin from 2026-30, with an additional c. 20bps of margin upside from these cost savings adding c. 1% upside to 2030 Core EBIT. **On the pipeline**, key Phase III readouts are expected in the 2028-30 period, starting with ris-rez (B7H3 ADC) in extensive small cell lung cancer, efimosfermin in F2/F3 MASH and velzatinib in 2L+ GIST, all of which face significant competition c. 2-3 years ahead. **On new launches**, Blenrep was £1m ahead and Exdensur was £2m below Consensus in 2Q, both still steady. **Overall, while we were encouraged by the self-financing of the acceleration in R&D and business development, we expect further clinical data across GSK's portfolio and evidence of launch execution will be required for upgrades to longer term forecasts given both Consensus and JPMe still remain below GSK's 2031 Sales target (JPMe 2031: £32bn pre Nuvalent).**

- **GSK reiterated their confidence in their >£40bn Sales 2031 target.** GSK reiterated their confidence in the 2031 >£40bn Sales target, with this target now factoring in the contribution from the recent Nuvalent transaction (see our take [here](#)), which we infer at least offsets the loss of camlipixant within the >£40bn target following the recent failure of the program. The >£40bn target does not depend on new business development from today. GSK also outlined their ambition to accelerate sales growth from 2031+, which we infer is underpinned by a combination of organic contribution from the current pipeline, supplemented by business development.
- **GSK also announced a restructuring program, with a "portion of cost savings" expected to support the Group margin through the dolutegravir LoE period.** GSK now expects to maintain a flat to improving margin through the dolutegravir LoE. While this was communicated during the Nuvalent transaction conference call, this has now been formalised as a target from 2028-30 with a portion of the annualised cost savings of £1.9bn by 2029 expected to support this. We expect the cost savings to start contributing to the P&L in 2028 (JPMe: £50m) with the full P&L benefit expected in 2029, which we estimate could be a c. £200m P&L benefit or a c. 60bps benefit in 2030. In our view, Company Consensus broadly already reflects this with a c. 40bps increase in Core EBIT margin from 2026-30, with an additional 20bps of potential margin upside from these cost savings adding c. 1% upside to Consensus 2030 Core EBIT.
- **FY26 narrowed to upper end of the range on Sales and Core EBIT, Core**

Underweight

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EPS narrowed to lower end with growth significantly 4Q weighted. GSK highlighted that they expect Core EPS growth to be significantly 4Q weighted given the inclusion of Nuvalent costs from 3Q, with 4Q 2026 growth benefitting from the supply chain and SG&A charges taken in 4Q 2025.

- **Key Phase III readouts expected in 2028.** GSK highlighted that they expect regulatory submissions for velzatinib (IDRX-42) in 2L gastrointestinal stromal tumours (GIST) along with ris-rez (B7H3 ADC) in 2L+ extensive small cell lung cancer and efimosfermin in F2/F3 MASH in 2028.

Oncology

- **Blenrep delivered £36m sales in 2Q 2026.** GSK noted that Blenrep sales of £36m in 2Q were in-line with their expectations and noted that in the US, their approach remains to “go slow” to ensure a positive experience for patients
- **B7H3 ADC Ris-rez data in extensive small cell lung cancer.** GSK outlined the previously presented 2L+ SCLC ARTEMIS-001 China-only data for ris-rez in extensive small cell lung cancer, which has led to the start of the Phase III EMBOLD SCLC-301 trial. In the Phase I ARTEMIS-001 trial, ris-rez demonstrated a 60% confirmed ORR, with 6.3 months median PFS and a 14.9 month median OS. This compares with Amgen’s* Imdelltra’s Phase III efficacy of 40% ORR and 13.6 month median OS, compared with 8.3 months median OS in the chemotherapy arm (HR: 0.60). The China-only Phase III for ris-rez recently reported a “clinically meaningful” Overall Survival benefit but we anticipate the full details of this study, potentially at ESMO 2026, will be needed to assess relative competitiveness vs. Imdelltra and whether this data can be reproduced in a global setting. We also note the Phase III EMBOLD SCLC-301 global trial includes patients previously treated with Imdelltra within the trial, which could dilute the ultimate Overall Survival result. Merck & Co’s I-Dxd* (B7H3 ADC) has also shown a strong ORR in this setting of 48%, with a median PFS of 4.8 months with a PDUFA for accelerated approval on October 10, 2026 and while I-Dxd was previously on partial clinical hold for Grade 5+ ILD events, the partial hold was lifted in January 2026. Overall, we see the 2L+ ESCLC indication as a modest opportunity for ris-rez as we see the high bar set by Imdelltra as difficult to beat and therefore see a 50% chance of a £0.5bn peak sales opportunity in this setting.
- **B7H4 ADC data in ovarian cancer and endometrial cancer highlighted.** GSK highlighted the data presented earlier this year on the B7H4 ADC Mo-Rez in 2L+ endometrial cancer and platinum-resistant ovarian cancer (see our take [here](#)). While we see the initial data as encouraging, we expect the FRa ADCs to take the majority share in platinum-resistant ovarian cancer, for which we expect Phase III data for Genmab in 4Q 2026, followed by Lilly’s sofetabart mipitecan in 2H 2027 and AZN’s torvu-sam in 2028 ahead of likely data for Mo-Rez in 2029. We also expect further mature data in endometrial cancer (GSK n=33 across two doses, AZN n=65, Rina-S n=66 across two doses), will likely be needed to assess the strength of the efficacy data relative to competition as the 2.8mg/kg dose showed a 9% ORR compared with 67% ORR in the 4.8mg/kg dose and we also note Grade 3+ TEAEs were 63% for GSK’s B7H4 ADC compared with 40% for AZN’s puxi-sam, where the Phase III readout in 2L+ endometrial is expected in 2H 2027.
- **Nuvalent update.** GSK continue to expect to file a supplemental NDA for 1L ROS for Jideytro (zidesantib) in 2H 2026 following the initial approval in 2L+ ROS mutated lung cancer last week. The ALKAZAR 1L trial is recruiting well with >35% of patients enrolled thus far and GSK noted that recruitment interest has increased following the presentation of the 1L data in TKI naïve patients at ASCO 2026. We continue to expect the Phase III ALKAZAR readout in 2029-30 (primary completion: Dec 2029).
- **Velzatinib data presented at ASCO highlighted.** GSK highlighted the velzatinib data

presented at ASCO (see our take [here](#)), where in 2L+ GIST, velzatinib showed a 38% ORR in 38 patients with a median PFS of 13.7 months, below the 16.5 month median PFS reported for bezucastatinib + sunitinib in the Phase III PEAK trial, although we note the control sunitinib arm showed a median PFS of 9.2 months in PEAK. We see a reasonable chance of success for the 2L+ GIST trial reading out in 2028 on the basis of this data, although expect the stronger efficacy for the combination approach, which could have a c. 2 year head-start over velzatinib, to lead to the combination strategy taking majority share. GSK highlighted the lower toxicity profile for velzatinib vs. bezucastatinib + sunitinib as offering another potential source of differentiation.

HIV

- **The Phase IIb CINERGY study of the VH-499 (capsid inhibitor) combination, a 2x-yearly HIV treatment, is expected to initiate in H2'26.** The INNOVATE Phase IIb for VH184 is ongoing data expected in 2027. GSK expect to start the Phase III trial for the combination of IM VH499 + IM VH184 (separately administered) in 2028, with targeted approval in the latter part of 2030. We note the primary endpoint for the Phase III trial is likely to be at 48 weeks, with time likely required for recruitment of the study as well so depending on the exact timing of the Phase III start in 2028, we anticipate the readout could be in 2030 but approval could be in 2031, which could lead to a relatively limited contribution from the 6M regimen in 2031.
- **The Phase III CUATRO study of the cabotegravir (INSTI) plus rilpivirine (NNRTI) combination, 3x yearly HIV treatment, was initiated in June 2026, with a primary endpoint of virologic suppression at month 11.** GSK are guiding for expected approval in 2028 though we expect this is likely towards the end of 2028 and contingent on recruitment pace. GSK continue to expect the majority of 2M Cabenuva patients to switch to 4M.
- **Weekly oral development started.** GSK has started a Phase I trial for VH359, a capsid inhibitor, and have multiple potential weekly oral integrase inhibitors and capsid inhibitors in preclinical development. GSK expect weekly orals to take a portion of the market in 2031. We note competitors GILD and MRK are ahead here with the recent positive lenacapavir/islatravir weekly oral Phase III ISLEND-1 and 2 data likely to crystallise weekly oral competition for Dovato, in addition to potential 2 drug regimen competition from BIK/LEN and DOR/ISL in 2027, although we expect a potentially more significant impact from the weekly oral. From our survey work ([here](#)), we see strong interest from physicians in the potential for a weekly oral, expected to take 18% market share longer term. GSK also outlined their expectation for the HIV treatment market to remain stable at c. £22bn in 2031 relative to 2025, with modest share expected to be taken by generic dolutegravir/Dovato orals, although we expect price pressure from the availability of generic Dovato and the use of generic Dovato will likely erode the value of the treatment market over time.

Respiratory, Hepatology and Vaccines

- **Exdensur sales of £18m in 2Q 2026 came in slightly below Consensus.** GSK noted that the J-code for Exdensur went live on July 1, with GSK continuing to build access and coverage with more than 50% of insured patients now covered. The majority of patient starts are from bio-naïve (56%), with 17% switches from other classes and 27% switches from IL-5. GSK expect bio-naïve to remain the majority of new patient starts going forward given asthma biologics penetration rates of 30%.
- **GSK highlighted the recent Phase III start for efimosfermin in F4 MASH and alcoholic liver disease (data likely in 2030), supplementing the trials in F2-F3 MASH (readout in 2028).** In MASH, the ZENITH-1 and ZENITH-2 trials in F2/F3 patients are actively recruiting, with primary completion in March 2028. GSK noted a potentially differentiated efficacy profile in F2/F3 MASH on the basis of a c. 24pp placebo-adjusted

improvement in fibrosis improvement, compared with a 20pp improvement for Novo's efruxifermin, although we note on the composite primary endpoint of fibrosis improvement of at least 1 stage and MASH resolution (the primary endpoint of the upcoming Phase III trials), efimosfermin showed a 21% placebo-adjusted benefit compared with a 36% placebo-adjusted benefit from efruxifermin. Novo's Phase III efruxifermin HISTOLOGY data in F2/F3 MASH is expected towards the end of 2026. We already include efimosfermin at a 50% chance of a £2bn peak sales opportunity, which assumes 30% share of the FGF21 class. GSK has also recently initiated the NEBULA-1 and NEBULA-2 Phase III trials in F4 MASH, which enrolled their first patient last week. Additionally, the efimosfermin development program has been extended into ALD, where the Phase III ALLSTAR trial is expected to start in H2'26. GSK reiterated efimosfermin's best-in-class profile for the treatment of MASH, highlighting data demonstrating the superiority of FGF21-class agents over non-FGF21 therapies in driving fibrosis improvement.

- **GSK also highlighted Phase III trial starts for their respiratory portfolio including their ULA TSLP (2x year administration) and quarterly IL-33.** GSK highlighted upcoming Phase III trial starts in 2H 2026 for the long-acting TSLP in COPD and nasal polyps and the long-acting IL-33 in 1H 2027, which we expect will likely readout in 2029/30 for the long-acting TSLP with the IL-33 data likely to follow in 2030/31. We see significant overlap in eosinophilic COPD between the TSLP (>150 cells per µl) and Exdensur (>300 cells per µl) for which we already model c. £4bn peak sales in both asthma and COPD, which we expect to be largely cannibalised by the long-acting TSLP depending on the strength of the data, where the Phase II data has not yet been presented.

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Date	Rating	Price (p)	Price Target (p)
31-Jul-23	UW	1383	1,300
02-Nov-23	UW	1423	1,400
05-Dec-23	UW	1455	1,450
01-Feb-24	UW	1568	1,530
02-May-24	UW	1705	1,660
02-Aug-24	UW	1526	1,550
31-Oct-24	UW	1407	1,440
07-Feb-25	UW	1480	1,500
01-May-25	UW	1484	1,460
04-Aug-25	UW	1396	1,400
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Source: Bloomberg Finance L.P. and J.P. Morgan; price data adjusted for stock splits and dividends.
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